Care Delivery Reform - Overview

A Two Layer Architecture for Universal Coverage

Introduction:

This is the Care Delivery Overview – one of four healthcare subdomains in the Futures Project Healthcare stack. The full stack covers care delivery reform, pharmaceuticals, transition mechanics, and our broader health improvement strategy. Healthcare reforms partially detailed here also connect directly to our **Labor**, **Fiscal**, **Agriculture**, **Housing**, **Economic**, and **Institutional Reform** stacks – following the platform's modular policy architecture whereby reforms in different domains are structured to reinforce the same underlying objectives. Modularity in this regard is designed to prevent the blockage of any one reform from creating cascading failures that ripple across other policy stacks.

But that being said, it is the firm position of this platform that a healthcare system that locks people into jobs for insurance, bankrupts families, and extracts rents at every layer is not just a health issue, but an economic and societal problem as well. Thus, healthcare reform and the policies surrounding it should be engineered to ameliorate these negative effects from every angle that they negatively affect the lives of the American people. As a disclaimer, this document gives the core argument and the key design decisions of changes proposed to care delivery in a broader healthcare reform package. It does not detail proposals for pharmaceutical reform, public health, or communication, and it is not a detailed spec on how to implement this reform from day-one. It is intended as a broad overview for how we can change care delivery – if more detail is needed, individual documents are available upon request.

How We Got Here:

The American healthcare system is by all accounts a mess. It is also a story of increasing complexity and opacity that was inadvertently built over the course of multiple decades. In order to fully understand how to fix our broken system we need to fully understand how it is broken. The easiest way to do so is to start with an insurance market that Americans intuitively understand, and then layer on the same fixes and decisions until it resembles the same maze we have in healthcare today.

Picture owning a car. For most day-to-day car ownership tasks (gas, oil changes, car washes, etc.) you pay directly. Some things cost more than others, but each item is shoppable. If someone tries to charge you ten dollars for a gallon of gas, you drive down the street and go somewhere else. For the things you genuinely cannot shop for (be that car accidents, natural disasters, etc.), you pay a monthly insurance premium –

that way, if something catastrophic happens, you are covered. The system is simple, the system is rational, and it works.

Employer Provided Healthcare – Post WWII:

Now imagine the cost of car ownership keeps rising and insurance is the biggest part of the bill. To fix this, the government starts giving your employer tax subsidies to cover part of your car insurance. This lowers your cost of car ownership, which feels like progress. But your employer quickly realizes that it can offer you good car insurance instead of higher wages — and that doing so could save them money and lower their tax bill. At first, you're fine with this setup because plans start competing to roll everything in – gas, oil changes, car washes, and all – and with a good job and a low monthly fee, it feels like you are driving your car for free.

The Connection to Healthcare: This mirrors the changes that happened to health insurance in the 1940s and 1950s. During WWII, Congress passed the Stabilization Act of 1942, which froze wages to prevent inflation.[1] As a result, employers started to offer health benefits to attract workers instead,[2] and this trend became really popular with voters. In 1954, the Internal Revenue Service (IRS) codified this by making employer-sponsored health insurance contributions tax exempt for employees and tax deductible for corporations,[3] which made the trend stick. And over the decades, this became the primary way that Americans received health insurance.

The Catch: Exploding Subsidies

But there is a catch to this system. The insurance company is no longer paying only for the rare, catastrophic things that insurance was designed for. Now it's starting to pay for every tank of gas, every car wash, and every oil change in the country. As a result, your rates go up, your employer's rates go up, and the subsidies from the government to your employer grow too. By our lifetimes this subsidy alone will reach nearly $300 billion a year[4] and is functionally the same as the government writing a check to your employer to help cover costs that only keep rising.

The Connection to Healthcare: By the late 1950's and early 1960's this structure caused two major shifts. Patients who weren't paying directly were consuming

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[1] AN ACT To amend the Emergency Price Control Act of 1942, to aid in preventing inflation, and for other purposes., Public Law 729, U.S. Statutes at Large 56 (1942): 765-768. https://www.govinfo.gov/app/details/STATUTE-56/STATUTE-56-Pg765-3.

[2] Federal Reserve Bank of Richmond, "From Loophole to Mandate: The History of Employer-Sponsored Health Insurance," *Econ Focus* 18, no. 1 (2014): [pg. 31], https://www.richmondfed.org/-/media/richmondfedorg/publications/research/econ_focus/2014/q1/pdf/economic_history.pdf.

[3] Internal Revenue Code of 1954, Pub. L. No. 83-591, sec. 106, 68A Stat. 32.

[4] U.S. Department of the Treasury, Office of Tax Analysis, Tax Expenditure Budget for Fiscal Year 2026 (Washington, DC: U.S. Department of the Treasury, November 27, 2024), https://home.treasury.gov/system/files/131/Tax-Expenditures-FY2026.pdf.

more care, and healthcare providers raised prices.[5] Today, the Joint Committee on Taxation estimates that the write off of employer-provided health insurance (from both income and payroll taxes) is the single largest federal tax expenditure, and costs nearly $300 billion annually.[6]

The Rise of Health Insurance Networks and Plan Micromanagement:

Meanwhile, the car wash figures out it can charge more to a nameless, faceless insurance company it has never met rather than to consumers directly. Prices start creeping up, and people without insurance are priced out. Over the decades, a car wash hits $200. A tank of gas hits $500. An oil change hits $2,000. The insurance company is aghast at these prices and responds by hiring entire departments to negotiate custom rates with every car wash, gas station, and auto shop in the country. The it starts building networks – this car wash is covered, that one is not. It adds deductibles – you pay the first $1,000 in gas yourself before coverage kicks in. Finally, it creates managed plans where staff micromanage every aspect of your car ownership and deny payment if you got an oil change without meeting the right conditions first. The car service companies hire their own staff to fight back, and all of that extra completely, extra staff, and extra cost gets priced back into your monthly rates, your employer's monthly rates, and the government subsidies to your employer to help you cover these costs. Prices explode.

The Connection to Healthcare: In order to combat prices in the early 1970's, Congress passed the **Health Maintenance Organization (HMO) Act of 1973**.[7] This opened the door for 'managed' healthcare – allowing insurers to invent networks, pre-authorizations, and utilization reviews in order to fight costs. Providers countered by hiring administration armies to fight the insurers, and – ironically – the costs of all of this extra staff just drove the cost premiums up further, increasing by over 500% from 1980 to 2000.[8]

Decades of Layering more 'Fixes' onto the Broken System:

And then we spend decades layering fix after fix on top of the broken system. We leave regulation to the states, insurers hire staff to comply with fifty different rule sets, rates go up. The government pays organizations directly for people who get in car accidents and cannot afford to pay, which costs tax dollars. We create government insurance for people who cannot afford private insurance, more tax dollars. We allow insurers and providers to merge on the theory that shared ownership will simplify billing,

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[5] Federal Reserve Bank of Richmond, "From Loophole to Mandate: The History of Employer-Sponsored Health Insurance," Econ Focus 18, no. 1 (2014): [pg. 34], https://www.richmondfed.org/-/media/richmondfedorg/publications/research/econ_focus/2014/q1/pdf/economic_history.pdf.

[6] U.S. Congress, Joint Committee on Taxation, Estimates of Federal Tax Expenditures for Fiscal Years 2023–2027 (JCX-47-23) (Washington, DC: Government Publishing Office, 2023).

[7] Health Maintenance Organization Act of 1973, Pub. L. No. 93-222, 87 Stat. 914.

[8] Kaiser Family Foundation (KFF). 2023 Employer Health Benefits Survey. San Francisco: KFF, 2023. https://files.kff.org/attachment/Employer-Health-Benefits-Survey-2023-Annual-Survey.pdf.

they negotiate with themselves and pocket the difference. We try more subsidies; they don't work well enough. By the time the 2020s arrive, the government is subsidizing your premiums and subsidizing your employer to cover your premiums, while taxing you to cover the people insurers say are too expensive to insure, and taxing you again to cover people who cannot afford insurance at all. Meanwhile your rates keep going up, your employers rates keep going up, and all of that money flows into a maze of billing staff, prior authorization teams, negotiators, auditors, and appeals departments hired to dispute and re-price the same services across every plan, for every employer, and every *personal* market plan, across fifty different regulatory environments.

The Connection to Healthcare: This maps to the series of band-aids that Congress used in an attempt to patch the system in the late 20th and early 21st centuries. Congress created Medicare and Medicaid in 1965 to cover the impoverished and the elderly who were left behind by the employer-sponsored model (who by that time and by that cost inflation, were effectively priced out).[9] Congress created the EMTALA Act of 1986 which forced emergency rooms to treat everyone regardless of their ability to pay.[10] The hospitals just turned this into higher prices for everyone else coming in the door. When costs kept rising, Congress passed the Affordable Care Act (ACA) of 2010 which layered on state-based exchanges and heavy federal subsidies to help affordability, but these did nothing to dismantle the underlying bureaucratic machine that was driving costs in the first place. Today, we spend more to utilize healthcare less,[11] and the price of these reforms have been largely passed to our premiums which have grown at nearly triple the rate of our wages over the decades.[12]

The Administrative Trench War:

To complete the car insurance analogy, what used to be a direct transaction – you pay the car wash; the car wash washes your car – has become an administrative trench war over who owes what to whom for services most of us could have afforded directly. This mirrors our healthcare spending, where we now fund parallel armies of administrative personnel to argue and haggle with each other over each line item to the tune of $5.3 trillion a year.[13]

For context: that is enough money to fund the U.S. military, plus the militaries of China, Russia, Germany, Japan, the United Kingdom, and the next thirty-five highest-

[9] Social Security Amendments of 1965, Pub. L. No. 89-97, 79 Stat. 286.
[10] Emergency Medical Treatment and Labor Act (EMTALA), 42 U.S.C. 1395dd (1986).
[11] Laura M. Keohane, Sunil Kripalani, and Melinda B. Buntin, "Traditional Medicare Spending on Inpatient Episodes as Hospitalizations Decline," Journal of Hospital Medicine 16, no. 11 (November 2021): 652–658, https://pmc.ncbi.nlm.nih.gov/articles/PMC8577699/.
[12] Salpy Kanimian and Vivian Ho, "US Medical Prices and Health Insurance Premiums, 1999-2024," JAMA Network Open 8, no. 12 (December 2025): e2547462, https://pmc.ncbi.nlm.nih.gov/articles/PMC12687089/.
[13] Shameek Rakshit et al., "How Has U.S. Spending on Healthcare Changed over Time?," Peterson-KFF Health System Tracker, January 22, 2026, https://www.healthsystemtracker.org/chart-collection/u-s-spending-healthcare-changed-time/#Per%20capita%20out-of-pocket%20expenditures,%201970-2024.

funded militaries in the world combined at their 2025 expenditure levels.[14] After all of that, we would have enough money left over to cover the entire GDP of Australia[15] and still have more than $400 billion left to spare.

The point of this illustration is not just to make light of a frustrating situation, but to show you that another change around the margins, another subsidy, another option, or repealing the latest subsidy will not fix this. We have tried these strategies for 70 years and failed. Every new patch created new complexity, which required new staff to manage it. Every new staff member required higher costs to insurers and providers. Every new cost eventually led to a higher monthly premium, or another subsidy by the government (which required more diverted tax dollars or created more debt (which leads to higher interest payments for the government). Meanwhile the system just got more complicated, inserted more people between doctors and patients, and delivered worse outcomes with every round.

The Plan

We cannot fix the broken system by layering another patch on top. Instead, we need a complete rebuild of the system from economic first principles. In this regard, there are two primary objectives: Make healthcare as affordable as possible and preserve as many choices as possible for the patient. This is not a plan to nationalize healthcare. Nor is it a plan to blindly hand the entire system to the private sector and pray for the best. Instead, we are using every structural, economic, and technological tool available in the 21st century to build a healthcare system where prices are visible, competition is real, and protection is universal.

Here is how that works in five steps:

Step 1: Firewall Shoppable Services and Emergency Care from Each Other

The most important structural decision in this reform is the line between two fundamentally different kinds of healthcare situations: Things you can shop for, and things you cannot (i.e. emergencies).

For all of the things you can shop for – be that primary care, scheduled imaging, dental cleanings, therapy appointments, routine labs, or even planned cosmetic surgeries, the platform strips out all of the middlemen and implements a direct to consumer market. This works because these things are predictable, schedulable, and comparable. When you need a service in this layer, you look up the price, you choose the provider, you book an appointment – and the market does its job. More importantly,

[14] Xiao Liang et al., Trends in World Military Expenditure, 2025, SIPRI Fact Sheet (Stockholm: SIPRI, April 2026), SIPRI Military Expenditure Database, https://www.sipri.org/sites/default/files/2026-04/2604_milex_2025.pdf
[15] International Monetary Fund, "Gross Domestic Product, Current Prices: Australia," *World Economic Outlook Database (April 2026 Edition)*, April 2026, https://www.imf.org/external/datamapper/NGDPD@WEO/AUS?zoom=AUS&highlight=AUS.

this has been piloted in the United States already, it has yielded costs that are substantially lower than going through traditional insurance.[16]

For all of the things you cannot shop for – be that cancer, a broken arm needing surgery, a heart attack, a serious accident, or anything where you are in an ambulance, unconscious, or facing a diagnosis that no household budget can absorb **–** the platform introduces **Severe Event Coverage**. This is a nationwide, universal, actuarially sound insurance floor that covers people during a crises. This is funded through monthly tax withholding (in place of the ~$1,440 in average employee premium contributions in 2025),[17] and it is reconciled as a tax credit for every month that people carry private insurance, so that no one pays for double coverage. Private insurers continue to compete above the **Severe Event Coverage** floor, but they can no longer deny care for basic emergencies. No deductibles, no networks, no games. If it's a severe event, it's covered. And you do not lose it by changing jobs, cutting your hours, or getting sick.

The firewall between these two categories is what makes both sides work. Routine care in the market means prices are visible and competition between providers is real. Severe events in the insurance layer means the national risk pool covers what it's good at covering – low-likelihood, high-cost events – instead of forcing insurance to pay every time someone needs a physical.

Step 2: Severe Event Coverage – The National Floor

When a severe event happens, you are covered. Lose your job in the morning and get in a car accident on the way home, covered. No deductible to hit first. No network to check. No gap because your employer stopped paying premiums last week.

Private insurance continues to operate above the floor. If your employer wants to offer a plan with better perks and lavender-scented air in the hospital, great. That is a real benefit above a real guarantee. But no one is negotiating over whether a heart attack for people over the age of 65 is covered. That question is already answered, regardless of where you work, what state you live in, how many hours you work, or what your employer decides to offer.

This emergency 'severe event' care is also regulated the same way that electricity, water, or fire stations are. Towns above a certain size need hospitals, just as they need fire stations and utilities, and the platform makes it happen. Additionally, anti-gaming architecture is layered in so that providers are called out if they start claiming that it costs them 5.8x more to do a surgery than any other hospital in their area. We do not force you to negotiate with the electric company when the power goes out, nor do we allow some private equity firm to charge you $100 dollars for every bucket of water a

[16] Fritz Busch, Dustin Grzeskowiak, and Erik Huth, *Direct Primary Care: Evaluating a New Model of Delivery and Financing*, Society of Actuaries Research Report (Schaumburg, IL: Society of Actuaries, May 2020), 5–8, https://www.soa.org/globalassets/assets/files/resources/research-report/2020/direct-primary-care-eval-model.pdf
[17] Rae et al., Kaiser Family Foundation. 2025 Employer Health Benefits Survey, 88.

fireman uses to put out a house fire. **Severe Event Coverage** extends this same logic to the healthcare system. The price of emergencies is regulated. The provider is still paid fairly. The difference is, now the public can finally see what emergency care actually costs.

With any healthcare reform system, there is naturally a question of how it will be funded. The **Severe Event Coverage (SEC)** system starts with the same underlying economics that insurance companies use to turn a profit every year. From there it makes the following changes. 1) Insurance companies naturally make larger profits by expanding the number of people that buy into their system – **SEC** coverage extends this logic to its maximum potential, pooling risk nationwide to make premiums as low as possible. 2) Insurance companies lose money on everyday consumptive healthcare expenses (i.e. people visiting their primary care doctor for normal checkups). **SEC** coverage pushes these everyday costs into the market layer and focuses on low-probability, high-cost events to keep premiums lower. 3) Insurers lose price negotiation power when they are small, **SEC** coverage seeks to pool a massive share of people and thus maximize its negotiation power to keep costs low. 4) Insurance companies expect to turn a profit every year, while baseline **SEC** coverage expects to break even and no more. 5) Finally, **SEC** coverage implements aggressive cost control measures on your behalf – simplifying rules and standardizing codes across the country to cut down on administrative costs, all the while negotiating without inserting itself between you and your doctor (These anti-gaming measures have their own section to explain their mechanisms in greater detail below).

As said previously, this is funded through monthly tax withholding (in place of the ~$1,440 in average employee premium contributions in 2025.)[18] This functions the same way that employee contributions to employer-sponsored health insurance works – with the funds automatically deducted on your paystub. If you choose to buy private health insurance instead, this private insurance is deducted as a tax credit for every month that you carry that insurance, so that no one pays for double coverage. For example, if you have private insurance from June till December, the taxes you would have paid for those months are credited back to you.

Step 3: The Market Layer – Transitioning from Employer Sponsored Coverage

In 2025, around 60% of Americans received healthcare coverage through an employer sponsored plan.[19] These sponsored plans cost employers an average of

[18] Rae et al., Kaiser Family Foundation. 2025 Employer Health Benefits Survey, 88.

[19] Gary Claxton, Matthew Rae, and Aubrey Winger, "Who Is Covered by Employer-Sponsored Health Insurance?," in Health Policy 101, ed. Drew Altman (Stockholm/San Francisco: KFF, April 15, 2026), https://www.kff.org/health-costs/health-policy-101-employer-sponsored-health-insurance/?entry=table-of-contents-who-is-covered-by-employer-sponsored-health-insurance

$7,884 (for single workers) / $20,143 (for families) [20] annually. Additionally, the average worker contributions to these plans were $1,440 for single coverage and $6,850 for families.[21]

The platform diverts these employer contributions ($7,884 / $20,143) into a **Healthcare Transition Account (HTA)** to cover your day-to-day costs**.** Instead of your employer spending time and resources negotiating custom terms for a bespoke insurance plan and *then* paying enormous sums of money every month to grant you the privilege of interacting with the healthcare system (which is often gated behind a high deductible anyways). Now those monthly contributions are diverted into an account using the same payroll software your employer uses for direct deposit. Additionally, this account is portable – if an employer has contributed $3,500 to your **HTA** so far this year and you change jobs, that $3,500 follows you wherever you go next.

These monthly contributions are *not for emergencies (which is covered by the **Severe Event Coverage** layer explained earlier).* They are for your everyday needs – primary care, direct dental membership models, vision, prescriptions, and so on. Direct primary care costs on average $65-85 monthly according to a 2020 review[22] – with 2025 prices showing a wider range of about $50-150 monthly.[23] [24] Adding additional plans for dental and vision is estimated to add another $25-50 monthly,[25] and these plans usually cover unlimited appointments, direct physician e-mail access, wholesale lab panels, imaging, and pathology at drastically reduced prices, immunizations, and more.[26] [27]For readers skeptical on the costs – even taking the most aggressively conservative estimates (which equate roughly $200 dollars monthly for primary care, primary dental, and primary vision combined) would yield around $2,400 annually. You could broadly triple that figure for good measure, and it would still yield an amount hundreds of dollars under the average **HTA** contributions proposed by this framework (which your employer typically pays for the sake of granting *access* to the healthcare system, and does not include deductibles, co-pays, or coinsurance costs).

This layer sits on top of the national **Severe Event Coverage** floor, which covers you for any emergency or ‘severe event’ regardless of how much money you have in your **HTA** account. If there is an emergency and you have to go to the hospital, you’re

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[20] Matthew Rae, Gary Claxton, Anthony Damico, Emma Wager, and Nirmita Panchal, 2025 Employer Health Benefits Survey (San Francisco: KFF, 2025), [84-88], https://files.kff.org/attachment/Employer-Health-Benefits-Survey-2025-Annual-Survey.pdf.
[21] Rae et al., 2025 Employer Health Benefits Survey, 88.
[22] Fritz Busch, Dustin Grzeskowiak, and Erik Huth, *Direct Primary Care: Evaluating a New Model of Delivery and Financing*, Society of Actuaries Research Report (Schaumburg, IL: Society of Actuaries, May 2020), 12, https://www.soa.org/globalassets/assets/files/resources/research-report/2020/direct-primary-care-eval-model.pdf
[23] Guilford-Blake, Roxanna. Wolters Kluwer, "The pros and cons of direct primary care (DPC)", accessed June 23, 2026, https://www.wolterskluwer.com/en/expert-insights/what-exactly-is-direct-primary-care
[24] Corsini, Christine. ColoHealth. "The Ultimate Guide to Colorado Direct Primary Care (DPC)." ColoHealth. Accessed June 23, 2026. https://colohealth.com/blog/colorado-direct-primary-care-guide/
[25] Pearly, "What Are In-House Dental Membership Plans?" The Dentistry Huddle, 2026, accessed June 23, 2026, https://www.pearly.co/dentistry-huddle/what-are-in-house-dental-membership-plans.
[26] Busch, Grzeskowiak, and Huth, *Direct Primary Care*, 63–64.
[27] "Membership in Pure Family Medicine," Pure Family Medicine, accessed June 26, 2026, https://www.purefamilymedicine.com/membership.

covered. If you lose your job, you're still covered. The market layer is for shoppable services only. The severe event coverage is also a floor. If your employer wants to buy a supplemental insurance on top of that to grant you access to luxury hospitals, or what-have-you – that is a legitimate benefit above a real floor, instead of the only thing standing between you and financial ruin if you get sick.

Step 4: Reconstructing Medicaid – Keeping the Market Layer Affordable

That **HTA account** contribution system might work for people *with* employer sponsored insurance, but it still leaves an open question on roughly 40% of Americans who do not qualify for employer sponsored care. Today, there are a growing number of 1099-style contract workers, gig workers, entrepreneurs, students, and so on that still need primary care health coverage. For these individuals, the market layer has affordability features built in.

Every primary care provider is required to submit their prices to a central platform where everyone can shop and compare services before they step out the door. And on the *patient* side of things, the system works with the **Labor Stack** of the Futures Project to provide an updated **benefits gradient** for healthcare. As a brief overview, the **benefits gradient** stack updates Medicaid, SNAP, and other government services to be a continuous slope-styled price discount using **Self Sovereign Identity (SSI)** and **Universal Payment Interface (UPI)** technology. Starting with the $200 monthly all-in primary care figure from earlier – as an example – patients would receive a flat percentage discount (e.g. 50%) at checkout. This discount system is calculated using two pieces of information: income, and pre-existing conditions. If you make $30,000 a year and have no existing conditions – you pay a discounted price. If you make $50,000 a year, but you're managing a chronic condition – you pay a different discounted price. If you're a veteran seeing a therapist – another discounted price. This discount accounts for both long-term care needs and income – and most importantly, it is structured so that there is no cliff whereby earning extra money suddenly causes you to lose your healthcare.

Critics will note that there is a common problem with Medicaid styled subsidies where prices drift upward over time to absorb whatever discount is being applied. When this happens, the subsidy intended to help the patient becomes a price floor the provider charges toward. Here the platform seeks to break that loop by making the discount visible to the patient, but not the provider. When you open the app and search for a primary care doctor, you see your price. When you walk in, the provider sees that the bill was paid, but they do not see what discount was applied or what your income is.

The logic behind this setup is that providers will not be able to price against a subsidy that they cannot see. This keeps the market honest, not through regulation alone, but because the information needed to game it is never transmitted in the first place. Importantly, this discount layer is for shoppable services only and is implemented on top of the **Severe Event Coverage** floor. If a gig worker or an entrepreneur gets

cancer, needs emergency surgery, or has some other severe event the national floor covers them regardless of their income, age, or pre-existing conditions.

Under this system, the *guarantees* of Medicare and Medicaid do not change. What changes is the delivery mechanism. Instead of fifty different state bureaucracies, separate enrollment portals, narrow provider networks, and staff whose only job is deciding who qualifies, the discount is shown to the worker in advance and applied automatically through the payment rail when the time comes. If successful, this platform transforms Medicaid to become a rule in the system instead of a maze for you and your doctor to navigate.

Step 5: Ensuring Data Privacy

Now the Medicaid restructuring might sound fancy, but for many people anything resembling a centralized government platform raises a lot of questions – and for good reason. Any healthcare system that does not respect the privacy of its patients is merely sacrificing one type of health for another. That is why all of this runs on a new privacy layer built around the patient (which connects to the Futures Project's broader **Self Sovereign Identity (SSI)** and **Universal Payment Interface (UPI)** policies as well as its **National Privacy and Security frameworks**).

Described briefly here: **SSI** and **UPI** are designed to trade the centralized legacy Social Security Number system (that was engineered around paper-based, non-random identifiers, and is known to have major security risks[28]), and replace it with a decentralized identity proofing system that allows users to verify individual attributes (like authorization, age, eligibility, or residence) using minimal disclosure frameworks that confirm only what is needed for a transaction (i.e. is this person over 21, yes or no?) without having to input raw information like Social Security numbers, Driver's License numbers, or medical histories online.

Under this platform, your information sits behind a secure digital vault commonly referred to as a secure enclave. When a provider, insurer, or payment system needs to verify whether you qualify for something, the vault returns a yes or no answer without exposing your private information. It can confirm that you qualify for a discount without showing your income to the doctor's office. It can confirm coverage without handing over your full insurance profile. It can settle a payment without exposing your medical history.

For healthcare to work, certain things have to be verified. The system needs to know whether you are covered, whether you qualify for a discount, whether a payment should go through, and what medical information your doctor needs to treat you. Today, that often means pushing your sensitive information to way too many institutions. All of them have to store that information in order to do their job properly, and each time this

[28] Electronic Privacy Information Center and American Civil Liberties Union, "EPIC and ACLU Comments on NIST's 2023 Digital Identity Draft Guidelines," *Electronic Privacy Information Center*, April 14, 2023, https://epic.org/documents/epic-and-aclu-comments-on-nists-2023-digital-identity-draft-guidelines/

happens, you reveal more information than you need to. Unfortunately, the result is that your medical records often live scattered across ten different doctors' offices, hospital systems, and insurance systems, and your personal information is left vulnerable if any of those centralized databases get attacked.

This new framework flips that dynamic entirely. The basic rule is simple: your doctor sees what they need to treat you. The payment system sees what it needs to complete the transaction. The eligibility system sees what it needs to confirm benefits. And no more. You see your records and control access. Nobody gets the whole file just because they touched one part of your care. And most importantly, your medical records follow you. You control who can access them, and the system is designed so nobody sees more than they need to do their job.

Anti-Gaming: Keeping Health Providers Honest

Every provider – every hospital, every surgery center, every clinic, and every doctor's office – is required to submit two things to a public platform: what they charge for every service, and what it *actually costs them to deliver it*.

Those codes are standardized, and the numbers are compared against every other provider in the country. Patients can see the price for a primary care visit at every practice in their area before they book. They can compare what a cosmetic procedure costs at different surgery centers. They can see where a provider's prices sit relative to the regional median. The price they see before they walk in is the price they pay. No surprise facility fees, and no surprise bill that arrives six weeks later and has nothing to do with what they agreed to.

By requiring that providers standardize their billing codes and submit their data to a central system, the government can benchmark how the cost to provide services in one town compares to demographically similar towns across the country. The technical term for this approach is called Euclidean nearest neighbors matching. In practice, this means that any provider who tries to cheat and randomly claim that a service costs them 5 times more to deliver than their peers is automatically flagged. Additionally, anyone who tries to fraudulently claim malpractice is flagged using a similar process. Both things cut down on major cost drivers for the old system. And this provides you with a market where prices are visible and provider compete (which is the opposite of what we have now).

The Real Benefits for Doctors:

Under this system, medical records are structured, consented, and follow the patient wherever they go. A physician no longer has to reconstruct an individual's medical history on the spot from memory, fragmented portals, faxed records, and whatever the patient can remember under stress. If successful, this reform will give doctors the data they need to diagnose and do their jobs effectively, without having to play detective for all of the underlying information in the first place.

Routine care is paid transparently through direct payment rails and the reformed Medicaid affordability layer. Catastrophic care is covered through **Severe Event Coverage** without doctors and patients fighting through fifty layers of insurer trench warfare. Once basic catastrophic protection is guaranteed nationwide, private insurers have to compete by offering real value above the floor rather than playing billing games, denying care, or sending patients surprise bills six months later. If insurers do engage in these practices, patients have far less reason to pay extra for them over the national floor. If successful, these reforms would cut down on billing games, allowing doctors to focus on their patients without having to worry about fighting third parties just to do their job.

Additionally, through the patient-doctor portal, doctors can charge a fair price for their services and see how their prices compare to those of other doctors in their region. These reforms allow doctors to bypass complex billing and insurance administration as well. This lowers costs and gives practitioners a stable way to predict what their revenue will be months in advance. The cost control measures are not in place to discriminate against a doctor who charges more than their peers; they are in place to prevent a hospital network from charging $50,000 for a procedure that costs them $3,500 to administer. For doctors specifically, they are free to charge what they want, and the market - not the government - will decide whether those services are priced fairly or not.

The same nearest neighbors matching frameworks that keep hospital networks honest on prices also apply to malpractice review. The system proposes using **SSI**, de-identified patient records, standardized clinical codes, and doctor review panels to evaluate malpractice claims against similar demographic and clinical cases, rather than litigating a case in hindsight. This reform proposes comparing an accused doctor's decisions with those made in similar medical situations and evaluates them using a jury-style panel of their peers, rather than forcing them to defend their career in a political or even punitive scenario. If successfully passed, tragic outcomes remain tragic, but a doctor's clinical decisions are evaluated by their peers using real-world data from conditions of uncertainty, rather than leaving their career to be judged by a courtroom that has never stood in their shoes.

If this platform is successful, it will have made a doctor's life easier. It will have reduced the risk of malpractice if decisions had reflected what a doctor would consider a reasonable decision in the face of uncertainty. It cuts down on administrative work and on fighting with insurance companies. And most of all, it allows doctors to charge a fair price for their services without the burnout that traditionally accompanies the old system.

Why this Platform is Not Medicare for All:

Many voters across the country have pushed for Medicare for All (M4A). To them, this platform may seem needlessly complicated, or even like a capitulation, when M4A could simply replicate the single-payer healthcare systems seen across the developed world.

They are right on efficiency. M4A would cover everyone, eliminate the insurer-provider billing war, and save massive amounts on administrative costs alone. It has been shown to work in capitalist economies all over the world, and it costs way less per capita than our current system does today.

But Medicare for All would also hand disaffected interests – like disgruntled insurers, hospital networks that can no longer charge astronomical rates -- and political incumbents the attack they have been warming up for fifty years. Even if the transition to *M4A was perfect* – if emergency room wait times and hospital closures were cut in half, and if the system somehow reduced nationwide deaths by 75% -- These incredibly wealthy and disaffected interests would have everything to gain by flooding the airwaves and social media with attacks saying that the government and its socialist medicine are to blame for endangering the American people.

In our current political and media environment, an ER wait time cut in half on one TV station becomes an ER catastrophe on another. Hospital closures cut in half on one podcast become an enumerated list of facilities that would still be open today if not for a reckless government takeover. Administrative efficiency gains become mass layoffs doomed to spread to other sectors of the economy as the government continues its slippery descent into Marxist ideologies. And finally, every unavoidable death in every hospital across the nation becomes blamable on the government and its dangerous experiment with socialism.

And that's what the attack surface is if the transition goes *perfectly*. The truth is that the transition will *not* be perfect. Implementing Medicare for All today would push the government to inherit the full cost and the full dysfunction of the current system on day one. Immediately, $5.3 trillion a year would move onto the books of a government that is already struggling financially (regardless of how taxes are raised to fund it). On top of that, the state would instantly inherit entrenched hospital consolidation games, a total lack of price transparency for routine services, failing maternal health outcomes, and a wave of rural hospital closures to fix all at once, without any of the specialized staff needed to manage them. To make matters worse, this entire transition would unfold while the government negotiates contracts with every healthcare provider in the country, while each one of these interests is structurally incentivized to undermine the reform at every opportunity.

None of these attacks would be fair. Half of them would seem laughable to doctors, nurses, or anyone who interacts with the healthcare system daily. But if the political discourse over the last two decades is anything to go by, these attacks will be believed by a solid proportion of the population regardless – and the fear that they instill will be real. As admirable as fixing healthcare is, it is not enough to deploy a good policy like M4A into a broken system, a hostile political landscape, and a captured media environment, and simply hope for the best. We need to fix the healthcare system in a way that will be structurally resistant to every political attack that will be thrown at it -- in a way that will resonates with skeptical voters in rural Nebraska, as much as it resonates progressive voters in New York.

This framework aims to do just that. Government takeover? This is the largest free market expansion in U.S. healthcare history. Killing private insurance? Private plans exist above the national floor – we're just forcing them to actually compete. Socialist medicine? This is cutting out the corporate middlemen and allowing people to pay their doctor directly for everything that makes sense. And if critics try to claim that the SEC Universal coverage is a socialist government takeover, our commercials write themselves: An insurance executive waking someone up in the middle of surgery to bargain the cost of an extra hour of anesthesia; an executive haggling with a patient in an ambulance and kicking them out over a $30,000 bill; or an everyday American paying their doctor directly while a disheveled politician shouts "socialism!" from the bushes outside.

Without a doubt, Medicare for All is a better system than what we have today. However, this system is designed to achieve the same outcomes as M4A through a mechanism that defangs the 'socialist medicine' attack entirely. This is not a pre-compromised strategy with the right; it's a calculated attack-surface decision informed by a sober look at the media environment that the policy must survive in.

Our two-tier design achieves universal coverage through two mechanisms. On the emergency side, it functions like single payer floor with a private insurance layer on top. On the primary care side, universal coverage is achieved by using income-adjusted pricing discounts through an updated Medicaid mechanism. To the skeptical voter who has been told for fifty years that socialism is a fast track to economic ruin, this direct-pay market functions as consumer choice rather than a government handout.

This platform wants the same things as the voters who want M4A. The mechanism it uses to achieve those outcomes might not be what they expect, but the underlying drive is unabashedly identical. The differences are that we're designing around the specific attack that's killed any serious healthcare reform for the last 70 years.

What Is not contained in this Document.

This is an overview document. Its purpose is to take the 8 policy briefs in the Care Delivery subdomain of the Healthcare stack and condense them into something someone can read in 10 minutes. Care delivery is but one piece of a larger healthcare reform that is needed in the United States. As such, it is worth mentioning that the each of the following will have their own documents, describing how the platform aims to tackle them in detail.

- **Other Care Delivery Reforms**: Dental care, maternal care, rural access, mental health, the full affordability gradient design, and the data privacy framework each have their own documents.
- **Pharmaceuticals** – drug pricing, recoupment-bound costs, PBM reform, public capital-backed drug discovery, and structurally reducing friction in Phase II and Phase III clinical trials – all have their own documents as well.

- **Transition Mechanics** – how we get from our current system to the new one, in what order, over what timeline, with what safeguards – have their own overview and their own documents.
- **Public Health and Nutrition** – food policy, nutrition standards, supplement industry reform, and making the food supply honest – have their own overview and documents as well.

The Bottom Line

The American healthcare system costs more than any comparable country, while simultaneously delivering worse outcomes. It bankrupts people and consistently fails us all when we are in our most vulnerable state. It is universally hated by doctors and the public writ large and remains absurdly complex for even the most mundane procedures.

None of this happened because of malice. It is not your doctors fault, nor your nurses fault. It is not the fault of the immigrant, or even the insurance CEO who only manages a single-digit profit margin every year. The system is broken because of seventy years of accumulated dysfunction, duct taping new layers and new complexity onto an already distorted system. Each new layer created new perverse incentives, and required new staff to manage it all, which ultimately created new costs for everyone involved.

This reform does not add another patch to the system. Instead, it strips the system down to its fundamentals and builds it back up from economic first principles. The new system is constructed to preserve your right to pick and pay for any primary doctor you choose. It is constructed to remove the middlemen and the cost inflation from the services you consume most. It is designed with affordability layers so that if you lose your job, or go back to school, or make another transition in life it adjusts accordingly. And most of all, it is designed to cover you, regardless of your financial situation in the case of an emergency.

Prices will be visible. Markets will be present, and universal coverage will exists where markets do not work. Your records will be your own, your Health Account will follow you wherever you go, and you will longer have to pay exorbitant sums of money for the privilege of not going bankrupt if you get sick. This reform will certainly not be perfect, but the bar is better than $5.3 trillion dollars a year disappearing into a black box while people go bankrupt anyways. That is not a high bar to hit, but it is a bar that we intend to clear by a large margin.

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